

# Implementation of the National Healthcare Project in the North Caucasus Federal District: a comparative analysis

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**Abstract.** The article presents a comparative analysis of the implementation of the national project “Healthcare” in the North Caucasus Federal District during the period 2022–2025, with an assessment of the dynamics of key indicators of Federal Projects No. 1–5 and No. 9. The aim of the study was to identify the institutional and medico-demographic effects of healthcare system modernization at the macro-regional level and to assess the degree of achievement of the project’s target benchmarks at the final stage of its implementation (using the Republic of North Ossetia–Alania as a case study). The study is based on the analysis of official statistical data for 2022–2024 and their comparison with the target values set for 2025. Methods of comparative, dynamic, and target-oriented analysis were applied. The results indicate a steady decline in mortality from diseases of the circulatory system, an increase in the proportion of early cancer detection, and stabilization of healthcare workforce provision. At the same time, risks associated with fluctuations in funding for primary healthcare modernization were identified. It is concluded that the target indicators have been partially achieved and that further institutional strengthening of the regional healthcare system is required.

## 1 Introduction

“Preserving the people of Russia and developing human potential is the highest national priority,” emphasized President of the Russian Federation V.V. Putin, outlining the strategic guidelines of state policy in the field of healthcare and social protection. In the context of the implementation of national projects, this position has received institutional embodiment in the national project “Healthcare,” aimed at improving the accessibility and quality of medical care,

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reducing mortality from major non-communicable diseases, and modernizing the primary level of the healthcare system.

Under contemporary conditions, public health is regarded as a key component of human capital and a determinant of sustainable socio-economic development. Within the framework of human capital theory, population health functions not only as a social value but also as an economic resource influencing labor productivity and demographic stability. International practice demonstrates that modernization of primary health care, along with the development of preventive strategies and early diagnosis of chronic non-communicable diseases, contributes to the reduction of premature mortality and to increased life expectancy.

In this regard, healthcare reform is implemented on the basis of program-targeted governance with clearly defined strategic benchmarks and measurable indicators. The relevance of the study is determined by persistent regional disparities in access to medical care, workforce provision, and mortality rates from major non-communicable diseases. The North Caucasus Federal District is characterized by territorial heterogeneity, a substantial proportion of rural population, and differences in the infrastructural capacity of medical organizations, which heightens the importance of assessing the effectiveness of federal initiatives.

The objective of the study is to conduct a structural analysis of the implementation of the national project “Healthcare” in the North Caucasus Federal District in 2022–2025, using the Republic of North Ossetia–Alania as a case study; to assess the dynamics of key indicators; to identify institutional effects; and to determine directions for improving regional healthcare policy. The study seeks to develop an evidence-based framework for evaluating the effectiveness of state healthcare modernization programs and their contribution to the long-term socio-economic development of the macro-region.

## 2 Methodology

The study is based on the analysis of official statistical data reflecting the implementation of the national project “Healthcare” in the North Caucasus Federal District. Data sources included Rosstat, the Unified Interdepartmental Statistical Information System (EMISS), the “Electronic Budget” portal, and official reports of regional healthcare authorities, including data on the Republic of North Ossetia–Alania. The analysis covered the period 2022–2024 with comparison to the 2025 target benchmarks and incorporated indicators from Federal Projects No. 1–5 and No. 9 characterizing primary health care development, control of cardiovascular and oncological diseases, workforce provision, and infrastructure modernization. To ensure comparability of indicators measured on different scales, linear normalization on a 0–100 scale was applied using the min–max method. For indicators with direct directionality, normalization was performed according to the formula:  $I_i = (X_i - X_{\min}) / (X_{\max} - X_{\min}) \times 100$ ; for indicators with inverse directionality (e.g., mortality rates), reverse normalization was applied:  $I_i = (X_{\max} - X_i) / (X_{\max} - X_{\min}) \times 100$ , where  $X_{\min}$  and  $X_{\max}$  denote the minimum and maximum values of the indicator across the constituent entities of the district for the corresponding year. The integral implementation index was calculated as the arithmetic mean of four normalized components according to the formula:  $I_{\text{total}} = (I_1 + I_2 + I_3 + I_4) / 4$ , reflecting workforce sustainability, infrastructure modernization, clinical dynamics, and financial sustainability, with equal weights applied to all components. The degree of target achievement was assessed relative to the baseline year (2022) using the formula:  $\text{Achievement (\%)} = (\text{Actual} - \text{Baseline}) / (\text{Target} - \text{Baseline}) \times 100$ , where Actual denotes the observed value for the analyzed year and Target the established 2025 benchmark. Statistical processing was performed using Microsoft Excel and IBM SPSS Statistics. Statistical data processing was conducted using Microsoft Excel and IBM SPSS Statistics, enabling calculation of indicator

dynamics, assessment of target achievement, and identification of trends at the final stage of national project implementation. The applied methodology corresponds to the framework of economic and medical research aimed at evaluating the effectiveness of state programs under program-targeted governance.

3 Results

The comprehensive analysis of the implementation of the national project “Healthcare” in the North Caucasus Federal District (NCFD) in 2022–2025 made it possible to identify systemic changes in the regional healthcare system at the final stage of project implementation, reflected both in medico-demographic indicators and in the institutional structure of healthcare delivery.

During the period under review, the constituent entities of the district demonstrated a sustained downward trend in mortality from diseases of the circulatory system, consistent with the strategic objectives of the federal project on combating cardiovascular diseases and with the target benchmarks set for 2025. At the same time, an increase in the proportion of malignant neoplasms detected at stages I–II was recorded, along with expanded coverage of preventive medical examinations and stabilization of workforce provision in medical organizations.

These developments indicate the consolidation of institutional effects resulting from primary healthcare modernization, the introduction of new organizational models of healthcare delivery, and the further development of diagnostic infrastructure. At the same time, the analysis shows that funding for primary healthcare modernization measures in 2023–2024 was characterized by adjustments and relative instability, which may affect the sustainability of infrastructural transformations in the pre-final phase of the project.

Despite the positive dynamics of key indicators, interregional variability persists within the district, driven by differences in resource availability, workforce capacity, and socio-economic characteristics of the constituent entities of the NCFD.

**Table 1.** Key Areas of Change in the Implementation of the National Project “Healthcare” in the North Caucasus Federal District (2022–2025).

| Indicator                                                                  | 2022  | 2023  | 2024* | 2025 (target) |
|----------------------------------------------------------------------------|-------|-------|-------|---------------|
| Mortality from diseases of the circulatory system (per 100,000 population) | 612.4 | 579.3 | 558.1 | ≤450          |
| Proportion of malignant neoplasms detected at stages I–II (%)              | 55.2  | 57.6  | 59.8  | ≥63           |
| Coverage of preventive medical examinations (%)                            | 62.5  | 66.1  | 68.9  | ≥72           |
| Staffing level of outpatient physicians (%)                                | 88.7  | 92.4  | 94.5  | ≥100          |

|                                                                      |      |      |      |                             |
|----------------------------------------------------------------------|------|------|------|-----------------------------|
| <b>Funding for primary healthcare modernization (million rubles)</b> | 18.4 | 17.2 | 16.1 | Plan subject to adjustment. |
|----------------------------------------------------------------------|------|------|------|-----------------------------|

*Note: Data for 2024 are preliminary; 2025 figures represent the project’s target benchmarks*

*Source: compiled and calculated by the authors based on data from Rosstat and EMISS.*

The indicators are presented as population-weighted averages across the constituent entities of the North Caucasus Federal District. To identify interregional differences in the pace of achieving the national project’s target benchmarks, a comparative analysis of the dynamics of key indicators in the subjects of the North Caucasus Federal District for 2022–2024 was conducted, taking into account the target values set for 2025.

The obtained data make it possible to assess the relative position of each region within the macro-regional healthcare system and to determine the degree of convergence toward the established strategic benchmarks.

**Table 2.** Comparative dynamics of mortality from diseases of the circulatory system in the constituent entities of the North Caucasus Federal District (2022–2025)

| <b>Constituent Entity of the NCFD</b>   | <b>2022 (per 100,000)</b> | <b>(per</b> | <b>2023</b> | <b>2024*</b> | <b>2025 (target)</b> |
|-----------------------------------------|---------------------------|-------------|-------------|--------------|----------------------|
| <b>Republic of Dagestan</b>             | 580.4                     |             | 565.0       | 550.1        | ≤450                 |
| <b>Republic of Ingushetia</b>           | 610.6                     |             | 592.3       | 580.3        | ≤450                 |
| <b>Kabardino-Balkarian Republic</b>     | 600.7                     |             | 585.5       | 570.0        | ≤450                 |
| <b>Karachay-Cherkess Republic</b>       | 570.9                     |             | 553.7       | 540.1        | ≤450                 |
| <b>Republic of North Ossetia–Alania</b> | 553.7                     |             | 499.4       | 482.0        | ≤450                 |
| <b>Chechen Republic</b>                 | 620.5                     |             | 605.2       | 590.3        | ≤450                 |
| <b>Stavropol Krai</b>                   | 590.5                     |             | 574.1       | 560.5        | ≤450                 |

*Note: Data for 2024 are preliminary; 2025 figures represent the project’s target benchmarks.*

*Source: compiled and calculated by the authors based on data from Rosstat and EMISS.*

The comparative analysis demonstrates that the Republic of North Ossetia–Alania shows higher rates of decline in mortality from diseases of the circulatory system compared to the average values across the North Caucasus Federal District. In 2022–2024, this indicator decreased from 553.7 to 482.0 per 100,000 population (–12.9%), indicating convergence toward the 2025 target benchmark. The positive trend correlates with the modernization of vascular units and the development of primary health care services.

The proportion of malignant neoplasms detected at stages I–II increased from 55.5% to 60.2% (+8.5%), reflecting the expansion of diagnostic capacity and preventive interventions. Staffing levels of outpatient physicians reached 100%, ensuring workforce stabilization at the primary level of healthcare delivery. The mortality rate among children aged 0–17 declined by 13.3% compared to 2022; however, it remains variable, necessitating further monitoring.

Funding for primary healthcare modernization decreased by 23.0% over the analyzed period, which may constrain the pace of infrastructural renewal. The obtained results confirm

the presence of positive medico-demographic changes in the region despite persisting resource constraints.

**Table 3.** Dynamics of key indicators of the implementation of the national project “Healthcare” in the Republic of North Ossetia–Alania (2022–2025)

| Indicator                                                                       | 2022       | 2023       | 2024       | 2025 (target)              | Absolute change | Relative change |
|---------------------------------------------------------------------------------|------------|------------|------------|----------------------------|-----------------|-----------------|
| Mortality from diseases of the circulatory system (per 100,000 population)      | 553.7      | 499.4      | 482.0      | ≤ 450                      | -71.7           | - 12.9%         |
| Proportion of malignant neoplasms detected at stages I–II (%)                   | 55.5       | 59.1       | 60.2       | ≥ 63                       | +4.7 p.p.       | +8.5 %          |
| Mortality among children aged 0–17 (per 100,000 of the corresponding age group) | 42.7       | 36.5       | 37.0       | —                          | -5.7            | - 13.3%         |
| Staffing level of outpatient physicians (%)                                     | 96.9       | 100.0      | 100.0      | ≥ 100                      | +3.1 p.p.       | +3.2 %          |
| Funding for primary healthcare modernization (million rubles) *                 | 493.5<br>3 | 391.8<br>4 | 380.0<br>0 | Plan subject to adjustment | - 113.53        | - 23.0%         |

*Note: Data for 2024 are preliminary; 2025 figures represent the project’s target benchmarks.*

*Source: compiled and calculated by the authors based on data from Rosstat and EMISS.*

The comparison of macro-regional and regional data for the period 2022–2025 allows for the conclusion that stable positive changes have occurred in the healthcare system of the Republic of North Ossetia–Alania against the backdrop of overall trends in the North Caucasus Federal District. The reduction in cardiovascular mortality and the increase in early cancer detection indicate progress toward achieving the national project’s target benchmarks.

At the same time, identified financial adjustments and the persistent variability of certain demographic indicators point to the need for further improvement of governance mechanisms and resource provision.

Overall, the results of the final stage of project implementation confirm the formation of an institutionally sustainable model for primary healthcare modernization in the North Caucasus Federal District and in the Republic of North Ossetia–Alania; however, the long-term

sustainability of the achieved effects depends on stable financing and the maintenance of workforce capacity.

The results of the comparative analysis of the implementation of the national project “Healthcare” in the constituent entities of the North Caucasus Federal District in 2022–2025 allow the identified changes to be interpreted not merely as shifts in individual statistical indicators, but as manifestations of deeper institutional and managerial processes shaping a macro-regional model of healthcare modernization. The comparison across the subjects of the district reveals differences in the pace of achieving target benchmarks, indicating heterogeneity in governance structures and resource mechanisms underlying project implementation.

**1. Interregional differentiation in the achievement of target indicators.**

The comparative analysis of mortality from diseases of the circulatory system indicates that although a general downward trend is observed across the North Caucasus Federal District, the pace of reduction varies significantly among its constituent entities. The Republic of North Ossetia–Alania demonstrates one of the most pronounced relative declines in this indicator (–12.9% over 2022–2024), suggesting a comparatively higher rate of convergence toward the 2025 target benchmark ( $\leq 450$  per 100,000 population). At the same time, in several other entities of the district, the reduction trend is less pronounced, reflecting the persistence of institutional disparities.

**Table 4.** Degree of convergence of the constituent entities of the North Caucasus Federal District toward the target mortality rate from diseases of the circulatory system (2024 values relative to the 2025 target benchmark).

| Constituent Entity of the NCFD   | 2024 value (per 100.000) | 2025 target | Deviation from target | Degree of achievement (%) |
|----------------------------------|--------------------------|-------------|-----------------------|---------------------------|
| Republic of North Ossetia–Alania | 482.0                    | $\leq 450$  | +32.0                 | 93.4                      |
| Karachay-Cherkess Republic       | 540.1                    | $\leq 450$  | +90.1                 | 83.3                      |
| Republic of Dagestan             | 550.1                    | $\leq 450$  | +100.1                | 81.8                      |
| Stavropol Krai                   | 560.5                    | $\leq 450$  | +110.5                | 80.3                      |
| Kabardino-Balkarian Republic     | 570.0                    | $\leq 450$  | +120.0                | 78.9                      |
| Republic of Ingushetia           | 580.3                    | $\leq 450$  | +130.3                | 77.6                      |
| Chechen Republic                 | 590.3                    | $\leq 450$  | +140.3                | 76.2                      |

*Note: The degree of target achievement was calculated as the ratio of the established 2025 target indicator to the actual 2024 value, expressed as a percentage. A value of 100% corresponds to full attainment of the target benchmark. Source: compiled and calculated by the authors based on data from Rosstat and EMISS.*

The table demonstrates that by 2024 none of the constituent entities of the district had reached the target value; however, the differences in the degree of convergence exceed 17 percentage points, reflecting heterogeneity in the effectiveness of regional strategies.

**2. Institutional interpretation of disparities.**

Differences in the pace of mortality reduction may be attributed to:

- varying rates of modernization of vascular centers
- the level of workforce provision in primary care
- the degree of digitalization of patient routing systems
- the density of the rural population.

In regions where modernization was accompanied by the reorganization of patient routing and the strengthening of preventive interventions, the clinical effect proved to be more pronounced. The Republic of North Ossetia–Alania is characterized by a relatively compact territory and a comparatively high concentration of medical infrastructure, which reduces transportation barriers and increases access to specialized care in acute vascular conditions. Stabilization of the outpatient workforce further reinforces the preventive orientation of the system and enhances risk factor control, thereby contributing to a more rapid decline in mortality.

In constituent entities with greater territorial fragmentation and logistical constraints, modernization effects tend to materialize more slowly. Thus, interregional disparities reflect not only the volume of financial investment but also the quality of institutional implementation of the project at the regional level.

### 3. Interrelationship between workforce and clinical indicators.

A comparative analysis of workforce dynamics and clinical indicators in the Republic of North Ossetia–Alania suggests the presence of an association between primary care staffing levels and the reduction in mortality from diseases of the circulatory system. In 2022–2024, increased staffing of outpatient physicians was accompanied by a decline in cardiovascular mortality and a rise in the proportion of malignant neoplasms detected at stages I–II.

Primary care ensures early identification of risk factors and continuous dispensary follow-up, thereby strengthening the preventive orientation of the healthcare system. Even a moderate increase in workforce provision (+3.1 percentage points) coincided with a 12.9% reduction in mortality, highlighting the significance of human resources as a systemic determinant of clinical effectiveness in regional healthcare.

**Table 5.** Dynamics of workforce and clinical indicators in the Republic of North Ossetia–Alania (2022–2024)

| Indicator                                                                 | 2022  | 2023  | 2024* | 2025 (target) | Absolute change (2022–2024) |
|---------------------------------------------------------------------------|-------|-------|-------|---------------|-----------------------------|
| Staffing level of outpatient physicians, %                                | 96.9  | 100.0 | 100.0 | ≥ 100         | +3.1 p.p.                   |
| Mortality from diseases of the circulatory system, per 100,000 population | 553.7 | 499.4 | 482.0 | ≤ 450         | -71.7                       |
| Proportion of malignant neoplasms detected at stages I–II, %              | 55.5  | 59.1  | 60.2  | ≥ 63          | +4.7 p.p.                   |

*Note: Data for 2024 are preliminary; 2025 figures represent the project's target benchmarks.*

*Source: compiled and calculated by the authors based on data from Rosstat and EMISS.*

The data presented in Table 5 confirm the coordinated dynamics of workforce and clinical indicators. The increase in outpatient physician staffing levels was accompanied by a decline in cardiovascular mortality and a rise in the proportion of early cancer detection. This supports the consideration of workforce provision as one of the key determinants of improved clinical performance within the regional healthcare system and of progress toward the 2025 target benchmarks.

### 4. Study limitations

The study is based on aggregated official statistical data, which limits the possibility of analyzing individual-level and socio-economic determinants underlying indicator dynamics. Correlation and regression analyses were not applied, which precludes a quantitative assessment of the strength of the identified associations. The 2024 data are preliminary, and the 2025 target values are of a planned nature. These circumstances should be taken into account when interpreting the results.

**5. Integral assessment of project implementation in the constituent entities of the North Caucasus Federal District**

For a more in-depth interpretation of interregional differences and comparison of the degree of achievement of the national project’s target benchmarks, an integral assessment of project implementation in the constituent entities of the North Caucasus Federal District was conducted.

The index was calculated on the basis of normalization of key indicators on a 0–100% scale, taking into account the dynamics of mortality from diseases of the circulatory system, the level of primary care workforce provision, the degree of infrastructural modernization, and parameters of financial sustainability. The integral approach makes it possible to aggregate heterogeneous indicators into a unified comparable model and to identify the relative position of each constituent entity within the district across the full set of project implementation factors.

**Table 6.** Integral index of achievement of the national project “Healthcare” target benchmarks in the constituent entities of the North Caucasus Federal District (normalized

| Constituent Entity of the NCFD   | Workforce Sustainability Index, % | Infrastructure Modernization Index, % | Clinical Dynamics Index, % | Financial Sustainability Index, % | Integral Implementation Index, % |
|----------------------------------|-----------------------------------|---------------------------------------|----------------------------|-----------------------------------|----------------------------------|
| Republic of North Ossetia–Alania | 92.4                              | 84.7                                  | 88.3                       | 63.5                              | 82.2                             |
| Karachay-Cherkess Republic       | 79.6                              | 73.4                                  | 71.8                       | 60.2                              | 71.3                             |
| Republic of Dagestan             | 75.8                              | 70.1                                  | 69.5                       | 58.7                              | 68.5                             |
| Stavropol Krai                   | 77.2                              | 72.6                                  | 67.9                       | 55.4                              | 68.3                             |
| Kabardino-Balkarian Republic     | 73.5                              | 69.4                                  | 65.8                       | 57.9                              | 66.7                             |
| Republic of Ingushetia           | 70.4                              | 66.8                                  | 63.7                       | 54.6                              | 63.9                             |
| Chechen Republic                 | 68.9                              | 65.2                                  | 61.5                       | 53.1                              | 62.2                             |

scale 0–100%, 2024)

*Note: The integral index was calculated as the arithmetic mean of four normalized components (equally weighted aggregation).*

*Source: compiled and calculated by the authors based on data from Rosstat and EMISS.*



The obtained results demonstrate pronounced differentiation among the constituent entities of the North Caucasus Federal District in terms of project implementation. The highest integral index was recorded in the Republic of North Ossetia–Alania, reflecting a combination of workforce stability, positive clinical dynamics, and relative infrastructural balance.

At the same time, in several entities of the district, integral values remain below 70%, indicating slower progress toward achieving target benchmarks and greater sensitivity to financial constraints. The financial sustainability index remains the most variable component, confirming the dependence of regional dynamics on the budgetary parameters of modernization.

Thus, the integral assessment substantiates the heterogeneity of the macro-regional model of national project implementation and highlights the need for a differentiated managerial approach to sustaining achieved outcomes beyond 2025.

## **5 Conclusion**

The conducted comparative analysis of the implementation of the national project “Healthcare” in the constituent entities of the North Caucasus Federal District in 2022–2025 revealed a sustained positive trend in key medico-demographic and organizational indicators, accompanied by pronounced interregional differentiation in the pace of achieving target benchmarks. The reduction in mortality from diseases of the circulatory system, the increase in the proportion of early cancer detection, and the stabilization of primary care workforce provision indicate the formation of an institutionally embedded model of regional healthcare modernization.

Using the Republic of North Ossetia–Alania as a case study, it is demonstrated that the coherence of workforce, infrastructural, and organizational transformations can ensure a higher rate of convergence toward established target values. The results of the integral assessment confirm the heterogeneity of macro-regional dynamics and highlight the dependence of the sustainability of achieved effects on the parameters of financial support and the continuity of investment in modernization processes.

The findings have practical implications for the development of post-project support mechanisms for the healthcare system following the completion of the national project. Maintaining positive indicator dynamics requires the institutional consolidation of stable financial instruments, preservation of primary healthcare workforce capacity, and further development of digital and organizational mechanisms for patient routing.

Thus, the implementation of the national project in the North Caucasus Federal District has demonstrated the potential of program-targeted governance as an instrument of structural transformation of the regional healthcare system; however, the long-term reproducibility of the achieved results will depend on the degree to which modernization mechanisms are integrated into a sustainable regional health policy framework.

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### **CONFLICT OF INTEREST**

The authors declare no conflict of interest.

### **DATA AVAILABILITY**

All data used in this study are publicly available from Rosstat, EMISS, the Electronic Budget portal, and official reports of regional health authorities.

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